

PATIENT DISCHARGE SUMMARY

St. Mary's General Hospital - Department of Internal Medicine
1500 Healthcare Parkway, Chicago, IL 60601

PATIENT INFORMATION

Patient Name: **Robert James Thompson**
Date of Birth: **March 14, 1958 (Age: 66)**
Medical Record #: **MRN-2025-087432**
Admission Date: **February 10, 2025**
Discharge Date: **February 17, 2025**
Length of Stay: **7 days**
Attending Physician: **Dr. Amanda Patel, MD, FACP**
Primary Insurance: **Blue Cross Blue Shield - Policy #BCB-445-2210**

ADMITTING DIAGNOSIS

- Acute exacerbation of Chronic Obstructive Pulmonary Disease (COPD) - ICD-10: J44.1
- Community-acquired pneumonia, right lower lobe - ICD-10: J18.9
- Type 2 Diabetes Mellitus, uncontrolled - ICD-10: E11.65
- Essential Hypertension - ICD-10: I10

HISTORY OF PRESENT ILLNESS

Mr. Thompson is a 66-year-old male with a 40-pack-year smoking history (quit 2019) who presented to the Emergency Department on February 10, 2025, with a 3-day history of progressively worsening dyspnea, productive cough with yellow-green sputum, fever (101.8F), and decreased oxygen saturation (SpO2 88% on room air).

The patient reports increased use of his albuterol inhaler over the past week (6-8 times daily vs. usual 2-3 times). He denies hemoptysis, chest pain, or recent travel. His last COPD exacerbation requiring hospitalization was in September 2024.

HOSPITAL COURSE

Day 1-2: Patient admitted to medical floor. Started on IV ceftriaxone 1g q24h and azithromycin 500mg q24h for CAP. Nebulized albuterol/ipratropium q4h. Supplemental O2 via nasal cannula at 3L/min. Prednisone 40mg PO daily initiated. Blood glucose monitored q6h with sliding scale insulin.

Day 3-4: Chest X-ray showed improvement in right lower lobe infiltrate. Temperature normalized. SpO2 improved to 93% on 2L O2. Transitioned to oral antibiotics. Blood glucose remained elevated (180-240 mg/dL); endocrinology consulted for diabetes management.

Day 5-6: Significant clinical improvement. Weaned to room air with SpO2 94-96%. Ambulatory without assistance. Endocrinology recommended adjustment of home diabetes regimen. Pulmonary function test performed: FEV1 52% predicted (baseline 58%).

Day 7: Patient stable for discharge. Tolerating oral medications. Ambulating independently. SpO2 95% on room air.

DISCHARGE MEDICATIONS

- Albuterol MDI 90mcg - 2 puffs q4-6h PRN shortness of breath
- Tiotropium (Spiriva) 18mcg - 1 capsule inhaled daily
- Fluticasone/Salmeterol (Advair) 250/50 - 1 puff BID
- Prednisone 20mg PO daily x 5 days, then 10mg x 5 days, then discontinue

5. Amoxicillin/Clavulanate 875/125mg PO BID x 5 more days
6. Metformin 1000mg PO BID (increased from 500mg BID)
7. Glipizide 10mg PO daily (new)
8. Lisinopril 20mg PO daily (unchanged)
9. Atorvastatin 40mg PO daily (unchanged)

FOLLOW-UP INSTRUCTIONS

1. Primary Care (Dr. Williams): Within 7 days of discharge
2. Pulmonology (Dr. Patel): 2 weeks post-discharge for PFT review
3. Endocrinology (Dr. Kim): 3 weeks post-discharge for A1C and medication adjustment
4. Chest X-ray: Repeat in 6 weeks
5. Labs: CBC, BMP, HbA1c in 2 weeks

Patient educated on smoking cessation maintenance, inhaler technique, and diabetes self-management. Home oxygen NOT required at this time.